



Physiotherapy & Rehabilitation Governance Platform

Medical Report: Post-ACL Reconstruction Rehabilitation

Patient Information

Name: ahmet kalumian

Birth Date: 2025-10-28

Case ID: #24

Clinical Details

Doctor: Dr. PSP

Date: 2026-03-28

Description: A 28-year-old male athlete, 4 weeks post-op for ACL reconstruction using a hamstring graft. Currently experiencing mild swelling and limited range of motion (ROM) in the right knee.

Physical Measurements

Height: 182 cm

Weight: 78 kg

Blood Group: OPositive

Health History

Smoker: No

Chronic Diseases: No

Activity Level: Active

Additional Patient Info

InjuryHistory: Complete ACL tear occurred during a football match on January 15, 2026, due to a sudden pivoting motion. Surgery was performed on March 1, 2026.

Medications: Ibuprofen 400mg as needed for pain, Vitamin D3 supplements, and Omega-3 fish oil.

FunctionalAbility: Partial weight-bearing with one crutch. Knee flexion is currently at 95 degrees. Struggles with stairs and long-distance walking.

PersonalGoals: Full return to competitive football within 9 months, restoration of full quad strength, and achieving 135-degree knee flexion.

Smart Clinical Analysis

Clinical Explanation:

The patient is a 28-year-old male athlete who is currently four weeks post-operative following an anterior cruciate ligament (ACL) reconstruction using a hamstring graft. He presents with mild swelling and limited range of motion (ROM) in the right knee, which is common in the early stages of recovery. The patient's functional ability is characterized by partial weight-bearing with the assistance of one crutch, and he is experiencing challenges with activities such as stair climbing and long-distance walking. His rehabilitation goals include a full return to competitive football within nine months, restoration of full quadriceps strength, and achieving 135 degrees of knee flexion.

Immediate Physiotherapy Recommendations:

The focus of physiotherapy should include gentle range of motion exercises to improve knee flexion, progressive strengthening exercises for the quadriceps and hamstrings, and modalities to manage swelling. Weight-bearing activities should be gradually increased as tolerated. Education on proper gait mechanics and the use of assistive devices should also be reinforced.

Precautions:

The patient should avoid high-impact activities, sudden pivoting motions, and any exercises that cause significant pain or discomfort. Care should be taken to monitor for signs of increased swelling or instability in the knee.

Estimated Recovery Timeline:

Given the current progress and rehabilitation goals, a full recovery and return to competitive football is anticipated within nine months, contingent upon adherence to the rehabilitation program and absence of complications.

Risk Assessment:

Potential risks include re-injury of the ACL, development of stiffness or adhesions in the knee joint, and delayed healing due to inadequate rehabilitation efforts. Close monitoring of knee swelling and pain levels is essential.

Red Flags:

Emergency warning signs include severe swelling, increased pain not relieved by medication, inability to bear weight on the affected leg, signs of infection (such as fever or redness), and any sudden changes in knee stability.

Test Group: Range of Motion (ROM) Assessment

Test: Knee Flexion (Right)

Clinical AI Analysis

Normal Reference:
Knee flexion typically ranges from 0 to 135 degrees in healthy adults.

Clinical Interpretation:
The patient currently demonstrates knee flexion measurements of 95, 100, 110, 110, and 115 degrees over the course of the recovery period. This indicates a positive trend in range of motion, although it remains below the normal reference range.

Progress Evaluation:
The trend shows consistent improvement in knee flexion, with a notable increase from 95 degrees to 115 degrees over the assessment period. This suggests effective rehabilitation efforts and responsiveness to therapy.

Physiotherapy Focus:
Continue to emphasize range of motion exercises, including passive and active-assisted flexion, while gradually introducing strengthening exercises for the quadriceps and hamstrings to support further gains in knee flexion.

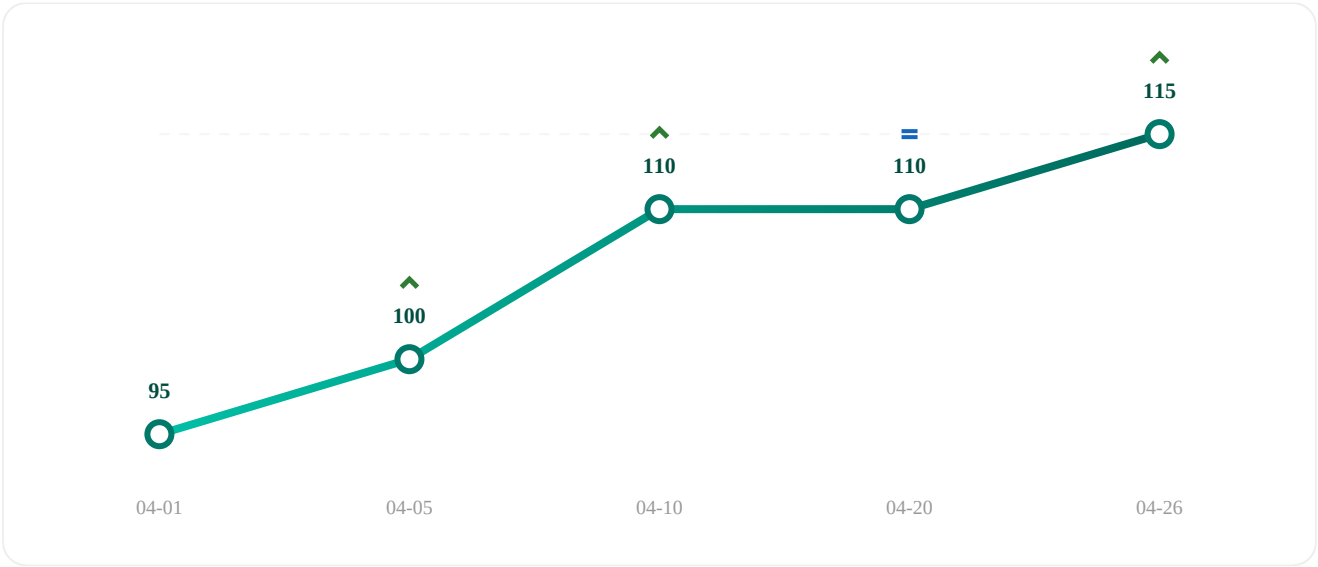
HIGHEST
115.0

LOWEST
95.0

AVERAGE
106.0

OVERALL PROGRESS

Increased by 21.1% ↑ Since first test (2026-04-01)



Date	Result
2026-04-01	95
2026-04-05	100
2026-04-10	110
2026-04-20	110
2026-04-26	115